

GULFSHORE VASCULAR & ENDOVASCULAR

412 Bayshore Boulevard, Suite 300 | Tampa, FL 33606 | (813) 555-0140 | Fax (813) 555-0141
NPI: 1497753102 | Tax ID: 59-5550142 | Fictional practice created for demonstration

PATIENT MEDICAL CHART

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1. Patient demographics

Patient name	Nora Alvarez
Date of birth	December 1, 1959
Age at visit	66 years
Sex	Female
MRN	PAD-35220
Address	55 West Shore Boulevard, Tampa, FL 33609
Phone	(813) 555-0118
Emergency contact	Elena Alvarez (daughter), (813) 555-0119
Primary care	Dr. Peter Huang, MD, (813) 555-0138
Preferred language	English and Spanish

2. Insurance

Primary insurance

Payer	Palm Mutual of Florida, Choice PPO
Member ID	PM66219044
Group	PM-TPA-22
Prior authorization	Required for peripheral revascularization
Auth telephone	1-800-555-0177

Secondary insurance. None on file.

3. Active problem list

ICD-10	Diagnosis	Onset	Status
I70.222	Atherosclerosis of native arteries of extremities with rest pain, left leg	2026	Active
I70.243	Atherosclerosis of native arteries of left leg with ulceration of ankle	2026	Active
I10	Essential hypertension	2012	Active

4. Current medications

Medication	Dose	Frequency	Indication
Clopidogrel	75 mg	Daily	Antiplatelet
Atorvastatin	80 mg	Daily	Lipids
Losartan	50 mg	Daily	Hypertension

5. Allergies

Allergen	Reaction	Severity
Iodinated contrast	Hives in 2018, premedicated since	Moderate

6. Visit notes — vascular surgery

Visit date: March 9, 2026 | **Attending:** Priya Raman, MD, FSVS | **Type:** New patient consult

Chief complaint. Left foot rest pain and a left ankle ulcer. She was referred for lithotripsy.

History of present illness. Nora Alvarez is a 66-year-old retired nurse. For one month she has had left forefoot pain at rest, worse at night, relieved by dependency. A 2 cm ulcer over the left lateral malleolus has been open for ten weeks. Pulses and the CTA tie the ischemia to a heavily calcified left superficial femoral stenosis. She does not smoke. She was not enrolled in a walking program because of rest pain and the open ulcer. A community cardiologist suggested shockwave lithotripsy because the plaque is calcified. She is here to schedule that procedure. She has no gangrene and no infection.

Examination.

Blood pressure 132/74 mmHg	Left foot Dependent rubor. 2 cm shallow ulcer at the left lateral malleolus. No bone exposed.
Pulses Left femoral present. Left pedal pulse absent.	Smoking Never smoker
Calcification Noted on the CTA report, described below	

Assessment and plan.

Critical limb ischemia of the left leg: ischemic rest pain and a nonhealing ischemic ulcer at the left ankle, attributed to the left superficial femoral stenosis.

A walking trial was not started. Critical limb ischemia is the indication.

The ordered technique is intravascular lithotripsy rather than a stent, because of dense calcification. Schedule HCPCS C9765.

Electronically signed: Priya Raman, MD, FSVS — March 9, 2026 at 2:15 PM

7. Diagnostic reports

Ankle-brachial index — February 26, 2026

Left ABI	0.48
Right ABI	0.98
Toe pressure	Not performed

Interpretation. Left ankle-brachial index is 0.48, which is below 0.90. The right index is normal.

Electronically signed: Priya Raman, MD, FSVS — February 26, 2026

CTA runoff — February 20, 2026

Indication	Left foot rest pain and ankle ulcer
Date	February 20, 2026
Contrast	Premedicated, no reaction

Interpretation. Iliac arteries are patent bilaterally. The mid left superficial femoral artery has an 85 percent stenosis with dense circumferential calcification over 5 cm. The popliteal artery is patent. Impression: 85 percent calcified stenosis of the mid left superficial femoral artery. The report describes calcification. It does not order a device.

Electronically signed: Owen Clarke, MD — February 20, 2026

8. Planned procedure

Planned procedure	Intravascular lithotripsy of the left superficial femoral artery
HCPCS	C9765 — intravascular lithotripsy, femoral or popliteal artery
Laterality and vessel	Left superficial femoral artery
Not ordered	CPT 37226 stent placement is not the order. The surgeon specifically chose lithotripsy.
Surgeon	Priya Raman, MD, FSVS
Payer	Palm Mutual of Florida
Authorization status	Not submitted. Request dated March 9, 2026.

Clinical indication summary

Critical limb ischemia with ischemic rest pain and a nonhealing left ankle ulcer.

Left ABI 0.48 on February 26, 2026.

CTA February 20, 2026: 85 percent stenosis of the mid left superficial femoral artery with dense calcification.

No walking trial, because the indication is critical limb ischemia.

The ordered code is C9765, intravascular lithotripsy, not a femoropopliteal stent.

This document is for clinical and administrative demonstration only. Patient identifiers are fictional.